

ASSESS

Patient-reported data, *without the chasing.*

Assess adds structured patient input to any orbit, from light check-ins to formalized eScreening and PROMs reporting, and it also runs entirely on its own, with no orbit required. Scores return structured, routed by role and urgency instead of waiting in a portal inbox. No app, no login, no new FTEs.



What Assess runs, three ways

- **Inside any orbit.** Symptom and progress check-ins scheduled to the clinical pathway, answered from the guide patients already open.
- **PROMs reporting programs.** Built for the pre-op and post-op survey windows that reporting programs actually enforce.
- **Standalone eScreening.** Validated instruments on schedule, no orbit required. Deployed across VA medical centers nationwide.
- **Routed, not parked.** Rising risk surfaced early, with scores routed by role and urgency instead of a portal inbox.

The PROMs mandate, in dates

CMS IQR, hip and knee first

- 01 Jul 2024: collection began.** Pre-operative PROMs capture required for elective hip and knee replacement. (CMS IQR / AAOS)
- 02 50%: required capture.** Minimum share of eligible patients with complete pre- and post-op surveys and risk variables. (CMS IQR / AAOS)
- 03 FY 2028: payment at risk.** Hospitals that miss reporting requirements face an annual payment reduction. (J. Arthroplasty, 2024)
- 04 2027: expansion arrives.** Mandatory reporting extends to outpatient and ambulatory settings after a voluntary phase. (J. Arthroplasty, 2025)
- 05 Completion that survives the gap.** Reminders ride the channel patients already respond to, protecting capture rates.
- 06 CPT-coded services supported.** Structured results into the screening services your clinicians already perform and document.

ESCREENING, PROVEN IN THE VA

Published two-cohort study, electronic versus paper screening

19+

VA medical centers adopted eScreening by 2022, 18 more in progress

VA Diffusion of Excellence; Pittman et al., 2017.

1,372

veterans in the published two-cohort study

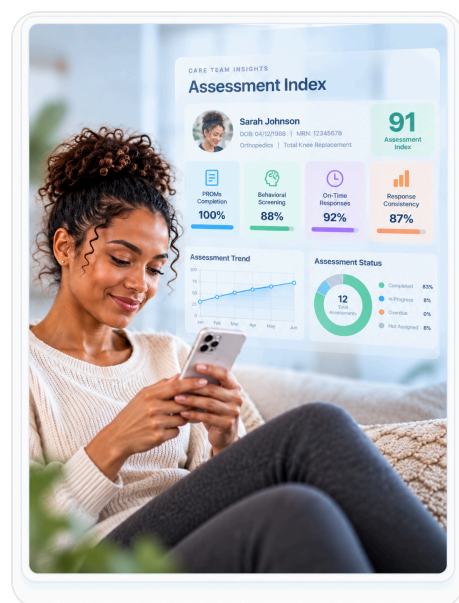
Higher

completion and access than paper screening

Faster

connection to care and suicide risk documentation

Pick one requirement. *Prove it in a quarter.*



An example Assessment Index, as your team sees it: every patient scored, routed by role and urgency.

STEP ONE: PICK THE REQUIREMENT

The screening or PROMs mandate you are on the hook for, and who reports it today.

- **Ortho PROMs.** The CMS hip and knee windows, pre-op and long post-op.
- **Behavioral screening.** PHQ-9, GAD-7, and the instruments your program runs.
- **In-orbit check-ins.** Light assessments woven into any journey.
- **Standalone eScreening.** No orbit required: an order, a list, a QR code, or a text.

STEP TWO: SCHEDULE THE INSTRUMENTS

Assess runs validated instruments on the required schedule, including the long post-op windows when patients are home and hardest to reach. Reminders ride the same channel patients already respond to, protecting the capture rates your reporting depends on.

STEP THREE: BUILT TO FOLLOW THE MANDATE

CMS has finalized expansion of PROMs reporting beyond inpatient hip and knee, to outpatient and ambulatory settings. Assess is built to follow it: ortho today, more service lines next.

REIMBURSEMENT AND FUNDING

Screening that pays its way

CPT codes shown as examples. Billing eligibility depends on payer policy and documentation.

- **Billable screening support.** Delivery and documentation behind CPT-coded services, such as 96127, 96160/96161, and G0444.
- **Grant initiatives.** Scheduled instruments and structured exports support grant-funded programs and community studies.
- **Capture rates that hold.** Reminders patients answer, so scheduled screenings are completed and billable, not missed.
- **Value-based reporting.** PROMs and screening results, structured for CMS, MIPS, and payer programs.

FOR THE PROGRAM

You can finally see it

- Completion rates that survive the post-op gap, by program and site
- Rising risk surfaced early, routed by role and urgency
- Structured results into your EHR, registry file, or team workflow

Engagement
Index Score
What is this?

91

Scores return to the dashboard, routed by role and urgency.

The next move: 20 minutes discussing *your* reporting requirements and *our* platform.

No demo script, no obligation. We hear which moments are costing your practice the most, and give you a brief look at the platform.

BOOK A 20-MINUTE INTRO CALL

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